Cisplatin, lomustine and vincristine (CLV)

CLV · 42 days NCCP 00806: selected adults with high-risk medulloblastoma or another eligible primitive neuro-ectodermal tumour, including pinealoblastoma. Use the exact prescription and schedule agreed with your specialist.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · CLV, hydration and antiemetic plan

Active medicines: Lomustine + Cisplatin + Vincristine

Emetogenic risk: high

Medicine	Dose / route	When
Individual high-risk antiemetic prevention and rescue	Agent / dose / duration to be completed by the prescriber and oncology pharmacist	Day 1 and delayed phase according to the agreed local regimen Incomplete review row; not a prescription. Pharmacy must reconcile aprepitant/CYP3A4 and vincristine interactions; do not auto-populate a generic aprepitant bundle.
Cisplatin hydration and electrolyte replacement	Agent / dose / duration to be completed by the prescriber and oncology pharmacist	Before and after cisplatin; reassess fluid status and renal function Incomplete review row; not a prescription. Complete fluids, rates, magnesium/potassium and monitoring for the actual product/local policy. No universal home fluid volume.
Constipation prophylaxis	Agent / dose / duration to be completed by the prescriber and oncology pharmacist	Start and continue on the individual vincristine bowel plan Incomplete review row; not a prescription. Select laxative and escalation instructions; review bowel symptoms before each vincristine dose.

Additional phase-specific support

- Cisplatin has high emetic potential; lomustine adds oral moderate-to-high risk. Use the team's prevention and rescue plan, including delayed nausea after Day 1. Aprepitant and some antifungals interact with vincristine, so the oncology pharmacist must select and check the plan; do not add medicines yourself.
- Cisplatin needs fluids before and after treatment and an individual electrolyte plan. Tell the team about fluid limits, swelling, heart or kidney conditions, vomiting and reduced urine. Follow your own home-drinking advice rather than a universal volume.
- NCCP recommends preventive laxatives with vincristine. Report constipation, new tingling, weakness, balance or hand-function changes before each visit. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent assessment.
- Have all medicines checked, including seizure drugs, steroids, antibiotics, antifungals, non-prescription products and supplements. The team will confirm the local antifungal prevention plan and choose medicines that avoid vincristine interactions. Other infection prevention follows the individual specialist plan.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-05 · SACTCheck v0.84.39 · Regimen-level drafts; prescriber and oncology-pharmacy verification required